

GATHER:

The Patient's Story on the Functional Medicine Timeline



JAMES CARTER, MD

APPLYING FUNCTIONAL MEDICINE IN CLINICAL PRACTICE

Disclosures

James Carter MD disclosed no relevant financial relationships with ineligible companies.

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Learning Objectives

At the conclusion of this segment, successful participants will be able to:

1. Explain the rationale for gathering oneself prior to each patient encounter.
2. Explain the features of the IFM Functional Medicine Timeline and its clinical utility as a point-of-care tool when gathering the patient's story.
3. Define the terms clinical findings, antecedent, trigger, and mediator as they relate to physiological dysfunction and clinical imbalances.

IFM Clinical Skills and Performance Objectives

THIS SEGMENT WILL ENABLE PARTICIPANTS TO DEVELOP & APPLY THESE SKILLS IN THEIR PRACTICE:

1. Perform a brief mindfulness practice to gather oneself prior to each patient encounter.
2. In addition to using intake questionnaires and other point-of-care tools, apply key questions to discover the patient's most important clinical findings, antecedents, triggers, and mediators.
3. Document the patient's clinical findings, antecedents, triggers, and mediators on the IFM Functional Medicine Timeline.

Clinical Focus

- Description and elements of the Gather step of the GOTOIT method.
- The practice of gathering oneself prior to each patient encounter.



The Functional Medicine Clinical Method

GOTOIT: The six steps of the functional medicine clinical method



GOTOIT

The sequence of steps used in the practice of functional medicine:

GOTOIT Step and Definition	IFM Tools (examples)	Clinical Actions
G – GATHER oneself with a brief personal practice prior to prepare for each patient encounter. Take a detailed functional medicine history that includes the essential health events and features of a patient's life documented on the IFM Timeline in the process of organizing the patient's data.	• Cultivating Self Awareness and Mindfulness • Adult Intake Form • Medical Symptom Questionnaire (MSQ) • PROMIS Questionnaires • Diet, Nutrition, and Lifestyle Journal • Functional Medicine Tree • Functional Medicine Timeline	• Use the Cultivating Self-Awareness and Mindfulness document to help gather yourself before every patient encounter. • The Adult Intake Form is used to gather initial intake information. Whereas, the MSQ, PROMIS Questionnaires, and Diet, Nutrition, and Lifestyle Journal help gather initial symptom, function, and dietary information. • Map root causes (antecedents, triggers, mediators (ATMs)) and effects (clinical findings, current concerns) of physiological dysfunction on the Functional Medicine Timeline.
O – ORGANIZE the data gathered in the previous step and document them on the IFM Timeline and Matrix to identify the personalized root causes of current concerns, dysfunctions, and conditions, and to begin to prioritize the order (sequence) of interventions.	• Functional Medicine Tree • Functional Medicine Timeline • Functional Medicine Matrix • Adult Intake Form • Medical Symptom Questionnaire (MSQ) • PROMIS Questionnaires • Diet, Nutrition, and Lifestyle Journal	• Using the Functional Medicine Matrix, populate ATMs, clinical findings, and current concerns from the Timeline. Add modifiable lifestyle factors (MLFs) and mental, emotional, spiritual factors (MES) to support the identification of the most important areas of physiological dysfunction.
T – TELL the life story back to the patient to ensure accuracy and a shared understanding. This dialogue, focused on the Timeline, highlights how factors elicited in the Gather step may have contributed to current or future clinical findings, concerns, health status and trajectory. The patient is asked to join in correcting the story, engendering a context of true partnership.	• Functional Medicine Matrix • Functional Medicine Timeline • Functional Medicine Tree	• The Functional Medicine Timeline is used to re-tell the patient's story while correlating prior events and root causes with current concerns or conditions. Ask the patient to join in for corrections or additional context. • Use the Functional Medicine Tree to explain how the root causes have contributed to the patient's condition.

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GOTOIT Step and Definition	IFM Tools (examples)	Clinical Actions
O – ORDER and collaboratively prioritize the most feasible and effective interventions by taking antecedents, triggers, mediators, and patient contextual factors into account.	• Functional Medicine Matrix • Readiness for Change • Goal Setting for Behavior Change • Integrating Guided Imagery and Visualization to Achieve Personal Health Goals	• Using the Functional Medicine Matrix, co-prioritize the areas of physiological dysfunction. • Assess if the patient is ready and committed to making changes with the Readiness for Behavior Change document. Use the Goal Setting for Behavior Change document to determine the patient's goals, values, preferences, and priorities. Then set SMART goals for interventions.
I – INITIATE Initiate interventions based on collaborative prioritization: 1) Evaluative, diagnostic, and prognostic testing. 2) Referral to another member of the collaborative care team for further assessment, intervention, or support. 3) Therapeutic interventions.	• Functional Medicine Tree • Functional Medicine Matrix • Functional Medicine Timeline • Goal Setting for Behavior Change • Functional Medicine Prescription and Lifestyle Plan	• The Functional Medicine Prescription and Lifestyle Plan helps layout the therapeutic plan. This may include removing or reducing triggers, modifying lifestyle and dietary mediators, addressing gut health and function, implementing other therapeutic interventions, ordering appropriate laboratory tests, or referring as needed.
T – TRACK and monitor the effectiveness of, and adherence to, the initiated interventions as well as interval life events and changes in the patient's circumstances.	• Medical Symptoms Questionnaire (MSQ) • PROMIS Questionnaires • Diet, Nutrition, and Lifestyle Journal	• Use tools to track progress by comparing scores at subsequent visits. The MSQ helps track symptom trajectory, the PROMIS Questionnaires help track overall well-being and quality of life, and the Diet, Nutrition, and Lifestyle Journal helps track adherence to regimen. • There are many other tools and questionnaires that can be helpful for tracking progress.

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Long Description

- I F M Toolkit Item: GO TO IT. Accessible version available in the course materials.

GOTOIT Functional Medicine Framework: GATHER



GOTOIT

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Long Description

- I F M Toolkit Item: GO TO IT framework with emphasis on gather.

Gather

Gather oneself with a brief personal practice to prepare for each patient encounter.

Take a detailed functional medicine history that includes the essential health events and features of a patient's life documented on the IFM Functional Medicine Timeline in the process of organizing the patient's data.

The GATHER Step of GOTOIT

GATHER ONESELF:

- Mindfulness; optimizing the therapeutic relationship.

GATHER INFORMATION:

- Through intake forms, questionnaires, the initial consultation, physical exam, and objective data.
- A detailed functional medicine history taken appropriate to age, gender, and nature of presenting problems.

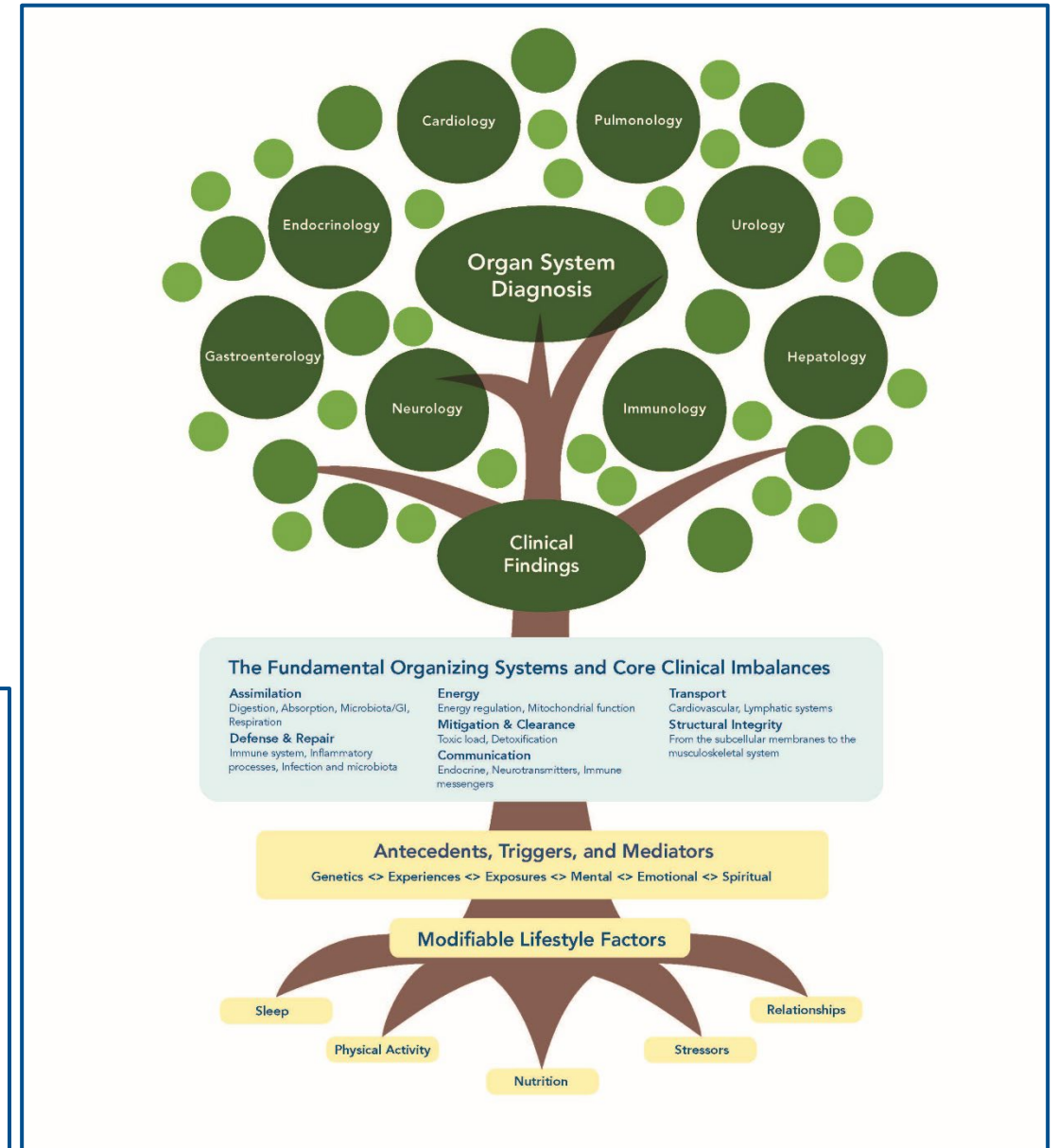
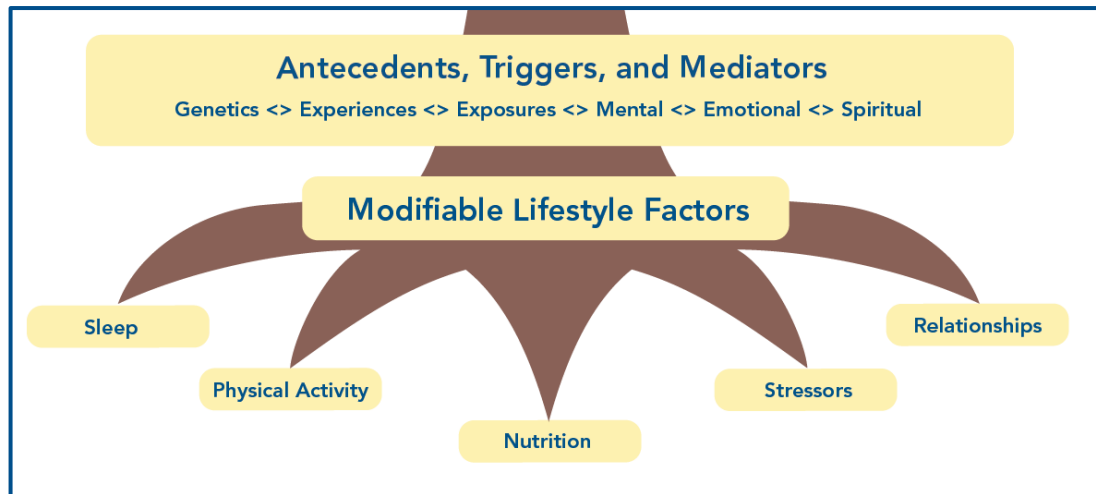
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Gathering the Story

Distinguishing Features That Inform the Functional Medicine Patient Evaluation



Long Description

- I F M Toolkit Item: The Functional Medicine tree with emphasis on the roots of the tree, including modifiable lifestyle factors, antecedents, triggers, and mediators. Accessible version available in the course materials.



How do you set the stage for a therapeutic encounter in clinical practice?

Setting the Stage for a Therapeutic Encounter



Self-gathering before
every encounter



Clinic environment



Special considerations for
telemedicine:

- Virtual backgrounds
- Audio and video quality



*Identify a brief ritual
(lasting only seconds)
that you intend to use
going forward to
gather yourself.*

Gathering Yourself



Gathering Yourself

Self-awareness is the process of paying attention to your emotions, needs, physical being, and reactions.

By becoming aware of the subtle, ongoing responses of the body, emotions, and mind, it is possible to detect patterns and take charge of your responses.



Cultivating Self-Awareness and Mindfulness

Self-awareness is the process of paying attention to your emotions, needs, physical being, and reactions. By becoming aware of the subtle, ongoing responses of the body, emotions, and mind, it is possible to detect patterns and take charge of your responses.

MINDFULNESS PRACTICES

- Breathing exercises
- Gratitude journaling
- Yoga, qi gong, and tai chi
- Guided imagery
- Mindful walking
- Meditation
- Many more! Try being mindful while eating, listening, or playing.

Understanding Self-Awareness

Awareness ranges from focusing on the present moment and your current physical, mental, and emotional state to understanding your history and future intentions. Being aware of your emotions and physical state allows you to choose how to respond. For instance, instead of letting physical discomfort push you into a grumpy state, you can acknowledge the discomfort and make a conscious choice to overcome it.

When many demands compete for attention or when we are tired, we often fall into rote patterns of emotion and behavior. We eat according to the clock, stay up too late, and react without thinking. We can become trapped in our minds or indulge in immediate gratification too often.

A lack of self-awareness can also lead to poor decisions, poor health, and poor relationships. Living on autopilot is a coping mechanism, yet conscious choices and decisions can dramatically improve your health and wellbeing.

Understanding Mindfulness

Mindfulness practices offer tools to anchor yourself in the present and can increase self-awareness. You can grow your ability to be self-aware and mindful—and every step you take will improve your health and happiness.

As you become more aware of your inner state, be compassionate with yourself. Recognize that there are many ways to practice mindfulness. Experiment to find what works best for you.

Practicing Mindful Observation & Reflection

Take a quiet moment and pay attention to your emotions. Are you feeling happy, angry, sad? Are your emotions reflected in or caused by the state of your body?

Long Description

- I F M Toolkit Item: Cultivating self-awareness and mindfulness handout. Accessible version available in the course materials.

Examples of Mindfulness Practices

- **Breathing Exercises:** Engage in deep inhales and slow exhales to center yourself.
- **Mindfulness During Daily Routines:** Engage in the present moment by observing your thoughts, feelings, and bodily sensations without judgment during normal daily activities (walking, handwashing, eating, etc.).
- **Meditation:** Calm your mind through techniques such as body scan, focusing on the breath, or chanting a mantra. This can be done for five minutes or less.
- **Gratitude Moment:** Take a brief pause to acknowledge and appreciate the positives in your life.

Examples of Mindfulness Practices

- **Mindful Listening:** Practice fully listening to your patients without planning a response.
- **Intention Setting:** Start your day by setting clear, positive intentions.
- **Visualization:** Use your imagination to both visualize the details and connect to the feeling of your positive intentions manifesting throughout your day.
- **Connect to Nature:** Spend a moment looking outside connecting with nature to ground yourself.

Importance of Mindful Meditation in Gathering Oneself

Flood of negative input through the amygdala decreases the creative and analytical parts of the cerebral portion of the brain.

Meditation decreases anxiety, calms the amygdala, increases our ability to think creatively and empathetically take others' perspective.

Birk M. Why leaders need meditation now more than ever. *Harvard Business Review*. Published March 22, 2020. Accessed May 30, 2025.
<https://hbr.org/2020/03/why-leaders-need-meditation-now-more-than-ever>

Empathic Listening in Practice

- Hearing the patient's story
- Present to **opportunity for meaningful inquiry and dialogue**
- Attend to **the patient's strengths**
- Attend to **the patient's context**
- Discover common ground



Gathering Oneself: User Experience

- **The Patient**

- Better satisfaction with practitioner
- Better health outcomes

- **The Practitioner**

- Mitigation of burnout
- Recovery of meaning, purpose, and job satisfaction

- **The Practice/Institution**

- Better morale
- Lower staff turnover

Clinical Focus

- The primacy of gathering oneself to mitigate implicit bias/unconscious bias.



Bias Definitions:

Bias: A particular tendency, inclination, positive or negative mental attitude, towards a person or thing.

Implicit Bias: Capable of being understood, even though it is not clearly or directly stated.

Explicit Bias: Clearly stated clearly in detail and intentional, leaving no room for confusion or doubt.

Unconscious Bias: Occurs without awareness, sensation, thought, or cognition.

Bias

The majority of our cognition is unconscious (System 1):

SYSTEM 1	SYSTEM 2
Fast	Slow
Unconscious	Conscious
Automatic	Intentional
Unreliable (error prone)	Reliable
Common for routine, repetitive tasks	Ideal for complex decision-making

Implicit Bias

- **IMPLICIT ASSOCIATION TEST** is a tool to identify personal biases of which we are unaware.
- **BIAS** expands beyond race.
- **UNCONSCIOUS BIAS** occurs despite well-meaning cerebral thoughts of empathy.

Implicit Bias and Health Disparities: Representative Examples

In a study of patients with uncontrolled hypertension, Blacks had shorter visits with their provider than Whites.

Sequist TD, Ayanian JZ, Marshall R, Fitzmaurice GM, Safran DG. Primary-care clinician perceptions of racial disparities in diabetes care. *J Gen Intern Med*. 2008;23(5):678-684. doi:10.1007/s11606-008-0510-7

The 31 Hispanic patients in this study were twice as likely as the 108 non-Hispanic White patients to receive no pain medication during the ED stay.

Cone DC, Richardson LD, Todd KH, Betancourt JR, Lowe RA. Health care disparities in emergency medicine. *Acad Emerg Med*. 2003;10(11):1176-1183. doi:10.1111/j.1553-2712.2003.tb00600.x

Mitigating Implicit Bias: Practical Strategies and Tactics

- https://www.aafp.org/pubs/fpm/blogs/inpractice/entry/implicit_bias.html
- <https://www.aafp.org/pubs/fpm/issues/2019/0700/p29.html>

GATHER: Part 2

The Patient's Story on the Functional Medicine Timeline



JAMES CARTER, MD

APPLYING FUNCTIONAL MEDICINE IN CLINICAL PRACTICE

Clinical Focus

- The functional medicine timeline: a point-of-care clinical tool to map the seminal health events of a patient's life.
- Clinical findings and current concerns on the timeline.

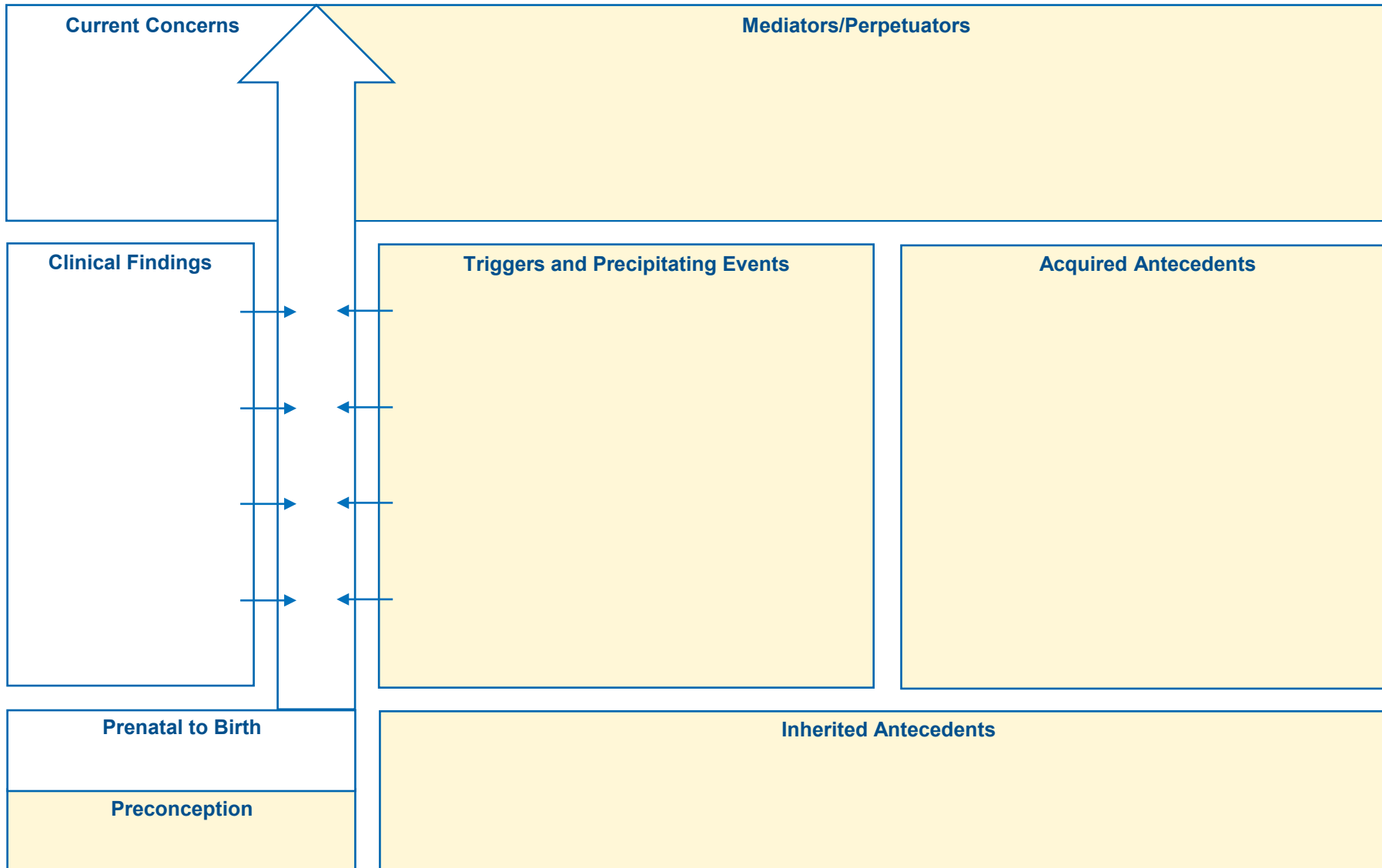


Gather

Populate Key Information on the Timeline



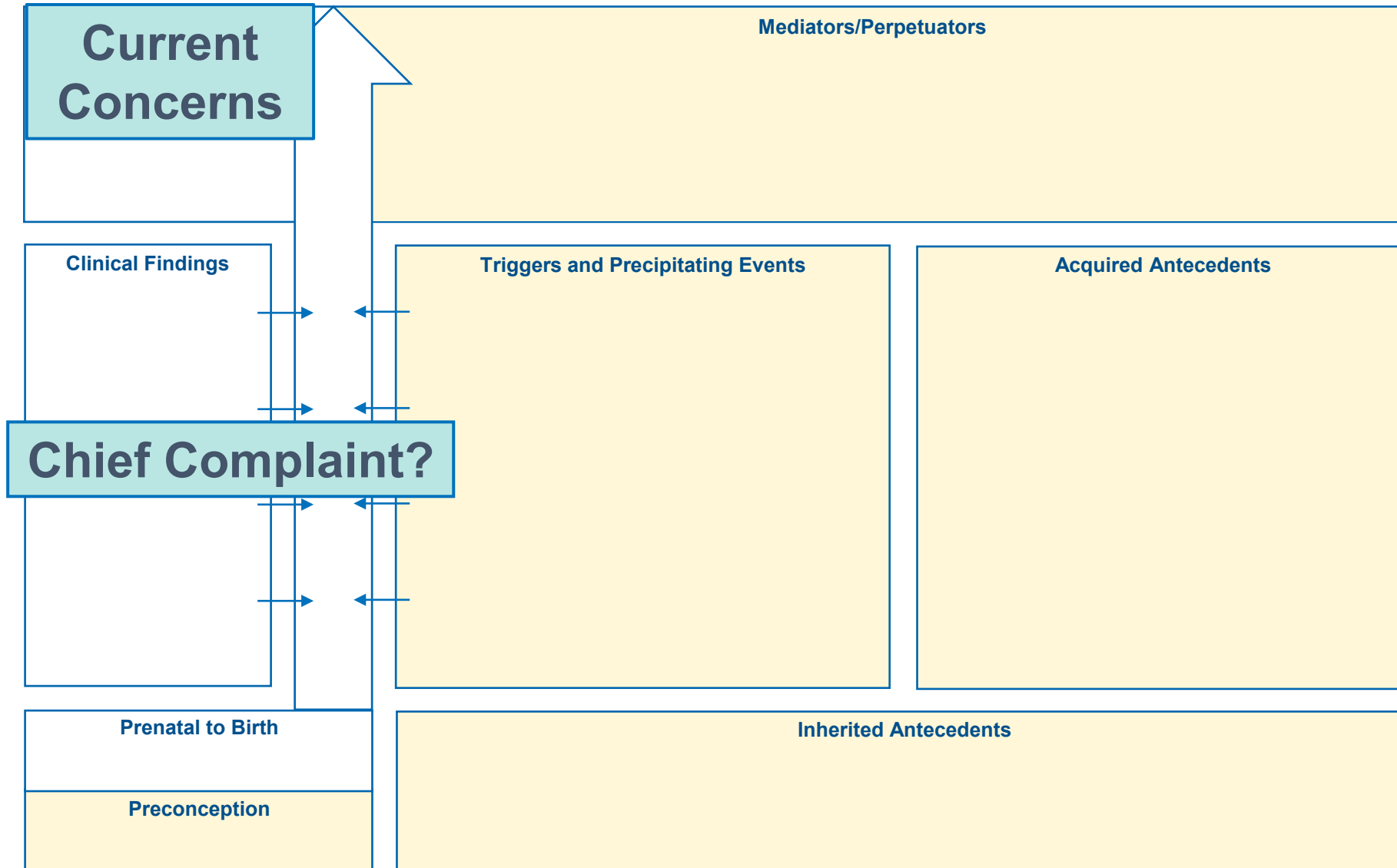
Functional Medicine Timeline



Long Description

- A blank Functional Medicine timeline.

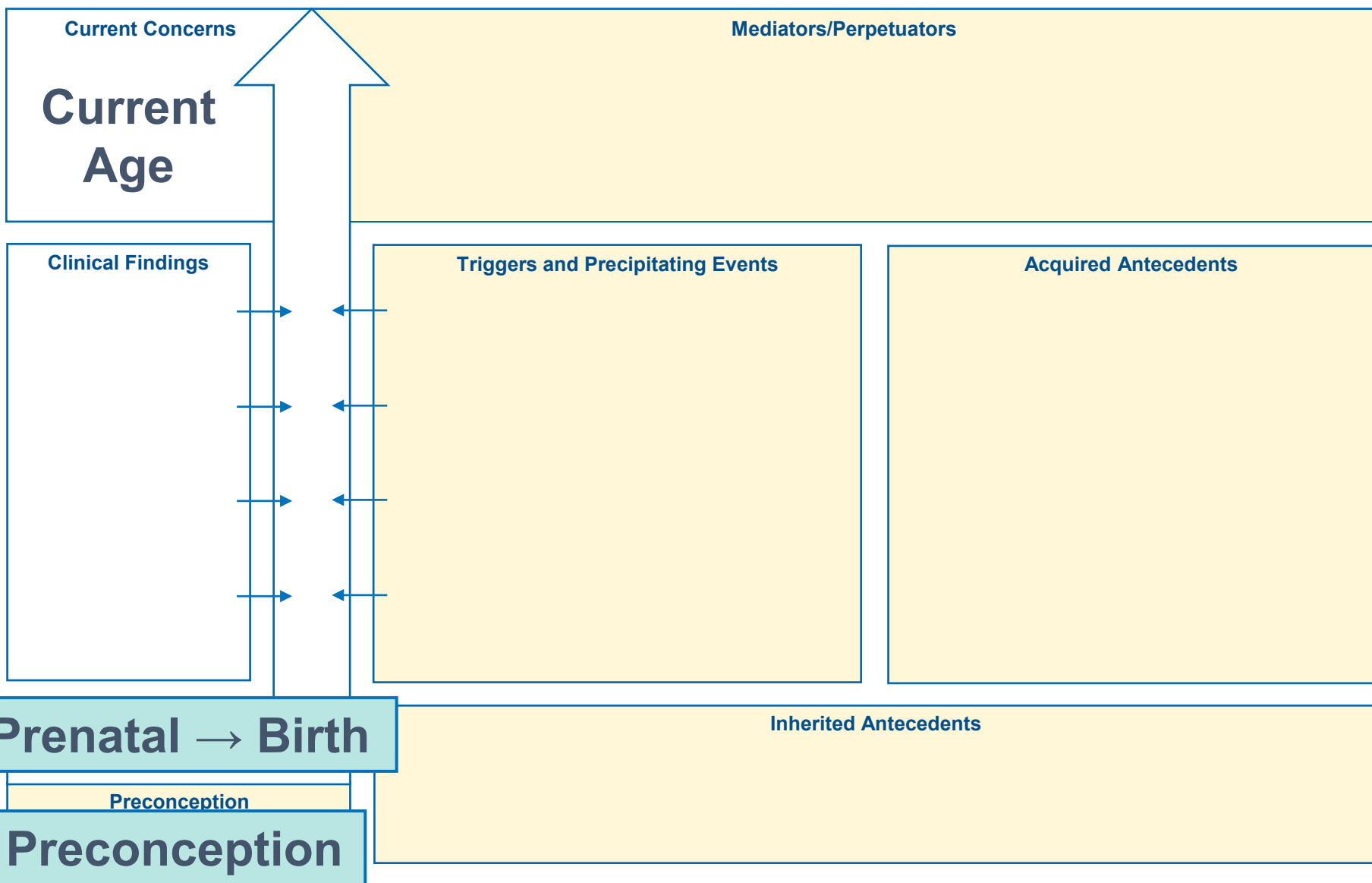
Functional Medicine Timeline



Long Description

- The Functional Medicine timeline with callouts of current concerns in the top left-hand side, and chief complaints overlaid on the clinical findings column on the left.

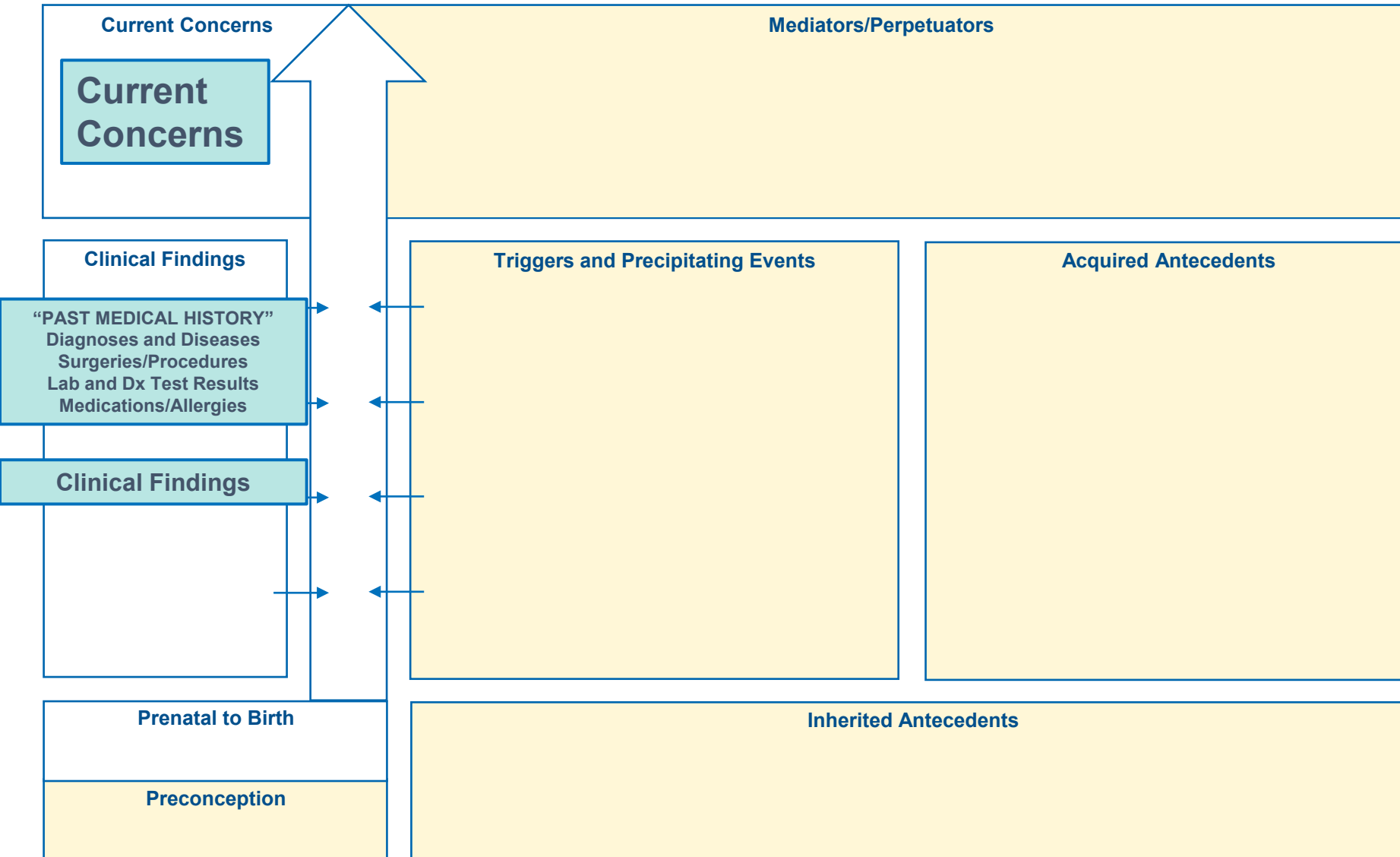
Functional Medicine Timeline



Long Description

- The Functional Medicine timeline with current age at the top, and emphasis on the prenatal to birth and preconception sections at the bottom of the timeline.

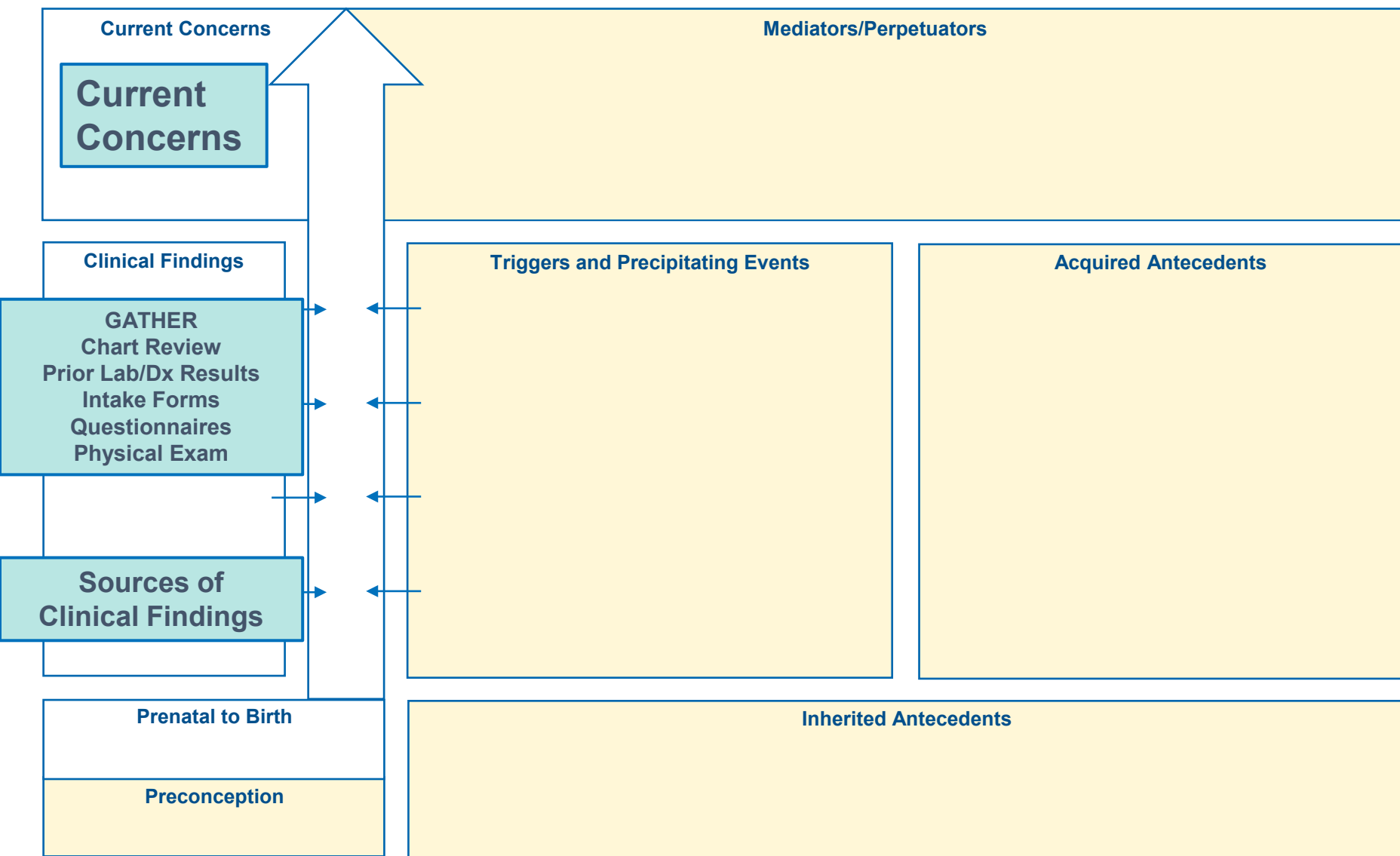
Functional Medicine Timeline



Long Description

- The Functional Medicine timeline with current concerns, past medical history, and clinical findings on the left-hand side of the timeline. Past medical history includes diagnoses and diseases, surgeries and procedures, lab and diagnostic test results, medications, and allergies.

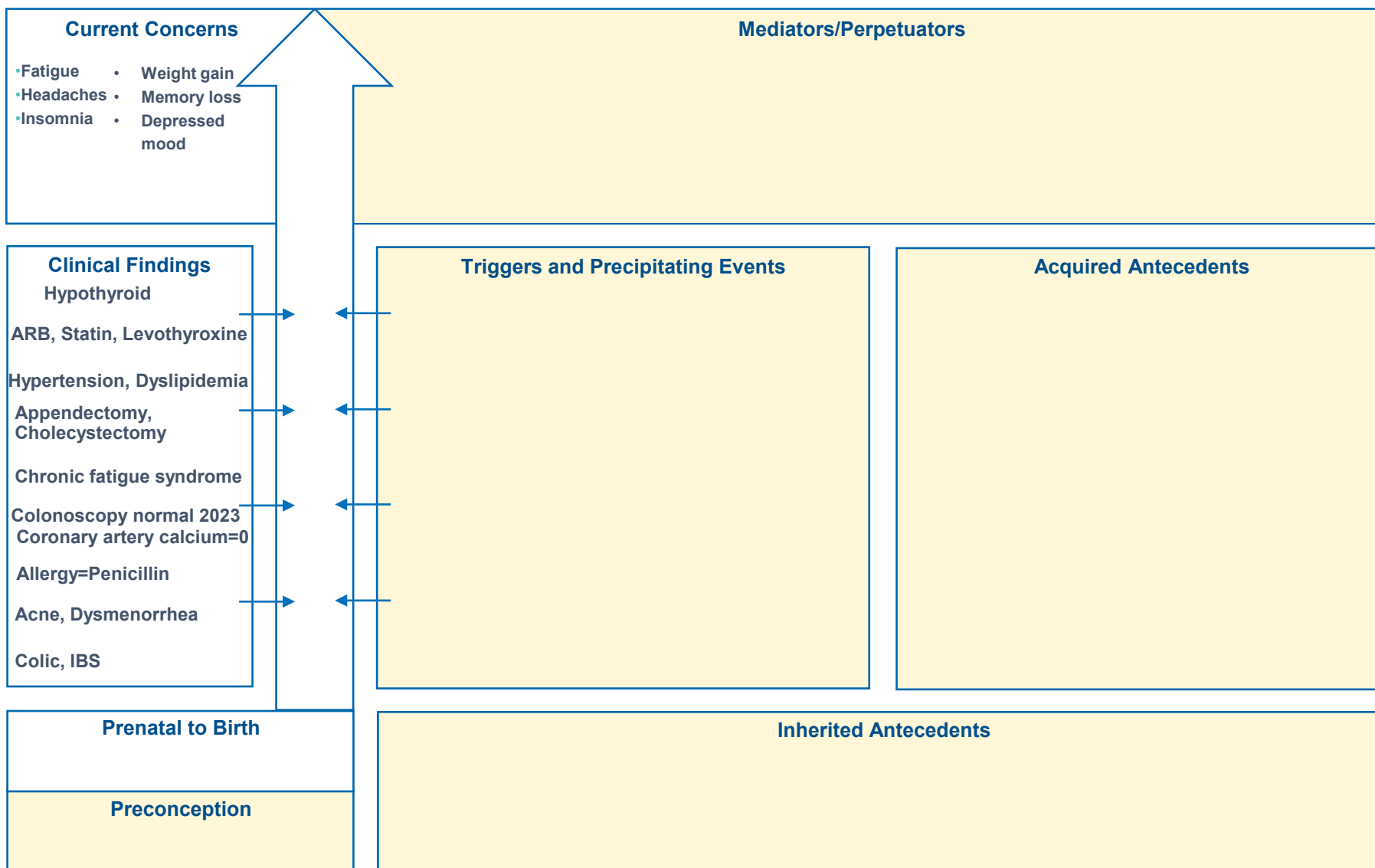
Functional Medicine Timeline



Long Description

- The Functional Medicine timeline with current concerns, gather, and sources of clinical findings on the left-hand side of the timeline. Gather includes chart review, prior lab and diagnostic results, intake forms, questionnaires, and physical exam.

Functional Medicine Timeline



**Examples of
Clinical Findings**

Long Description

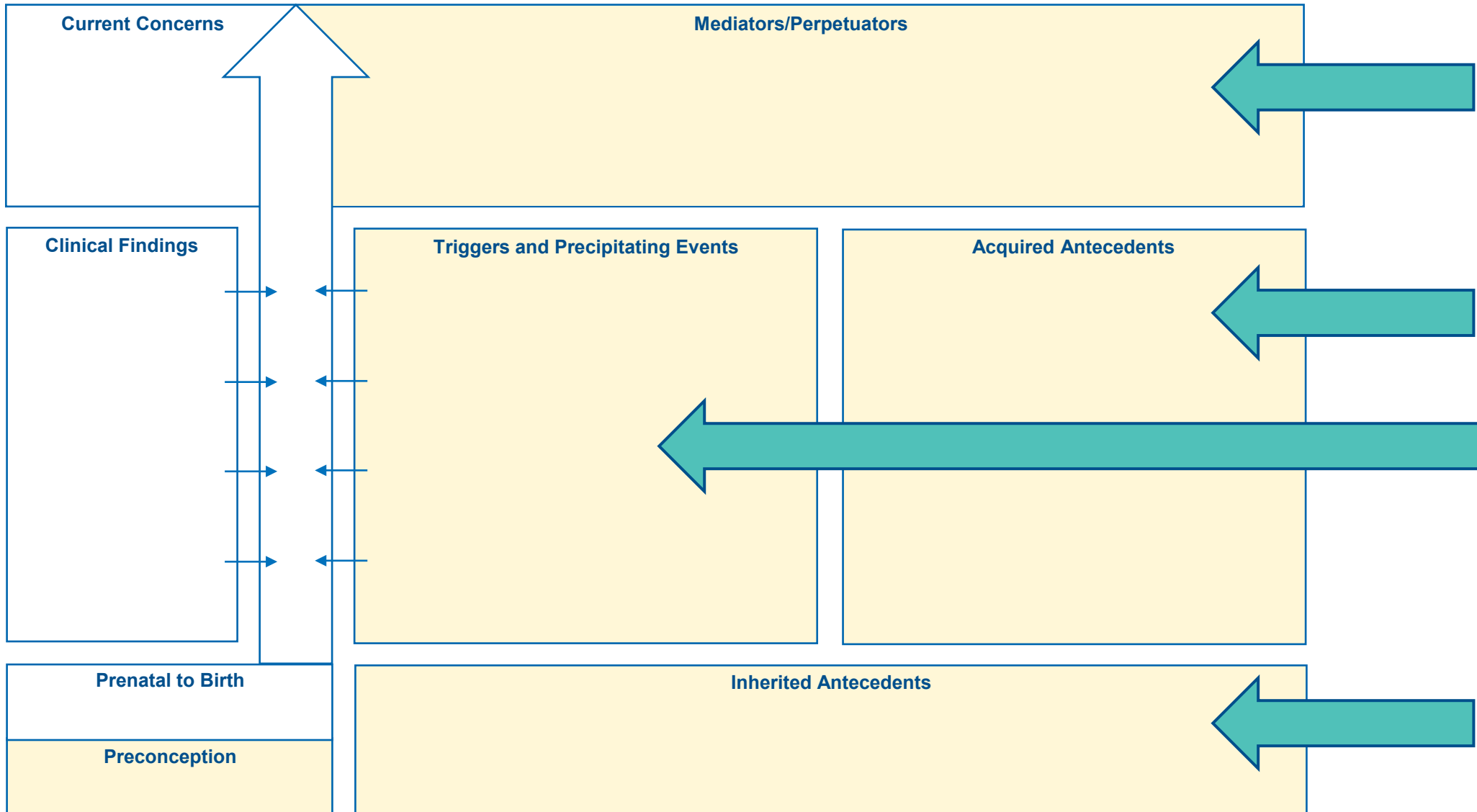
- The Functional Medicine timeline with the current concerns and clinical findings sections filled out.
- Current concerns
 - Fatigue, headaches, insomnia, weight gain, memory loss, depressed mood.
- Clinical findings listed from most recent to older.
 - Hypothyroid
 - A R B, statin, levothyroxine
 - Hypertension, dyslipidemia
 - Appendectomy, cholecystectomy
 - Chronic fatigue syndrome
 - Colonoscopy normal 2023
 - Coronary artery calcium = 0
 - Penicillin allergy
 - Acne, dysmenorrhea
 - Colic and I B S

Clinical Focus

- Definition of Antecedents, Triggers, and Mediators.
- Antecedents, Triggers, and Mediators on the Timeline.



Functional Medicine Timeline



Long Description

- The Functional Medicine timeline with arrows highlighting the following sections: mediators/perpetuators, triggers and precipitating events, acquired antecedents, and inherited antecedents.

Antecedents

- Antecedents are any factors, inherited or acquired, that predispose or confer susceptibility to a disease, diagnosis, or dysfunction.
- The effects of antecedents on physiological function, clinical findings, and health outcomes are usually delayed by months to years.

Examples of inherited antecedents:

Family history of “x”

ApoE4 allele

In utero milieu/environment

Inherited Antecedents

Therapeutic interventions whose mechanism of action is to modulate the expression of genes are referred to as “epigenetic interventions.”

Some inherited antecedents may be modulated by epigenetic interventions (and by epigenetic environmental exposures and trauma).

Inherited Antecedents

Examples of interventions that may modulate inherited antecedents via epigenetic effects:

- **Targeted dietary nutrients**
- **Stress reduction practices**

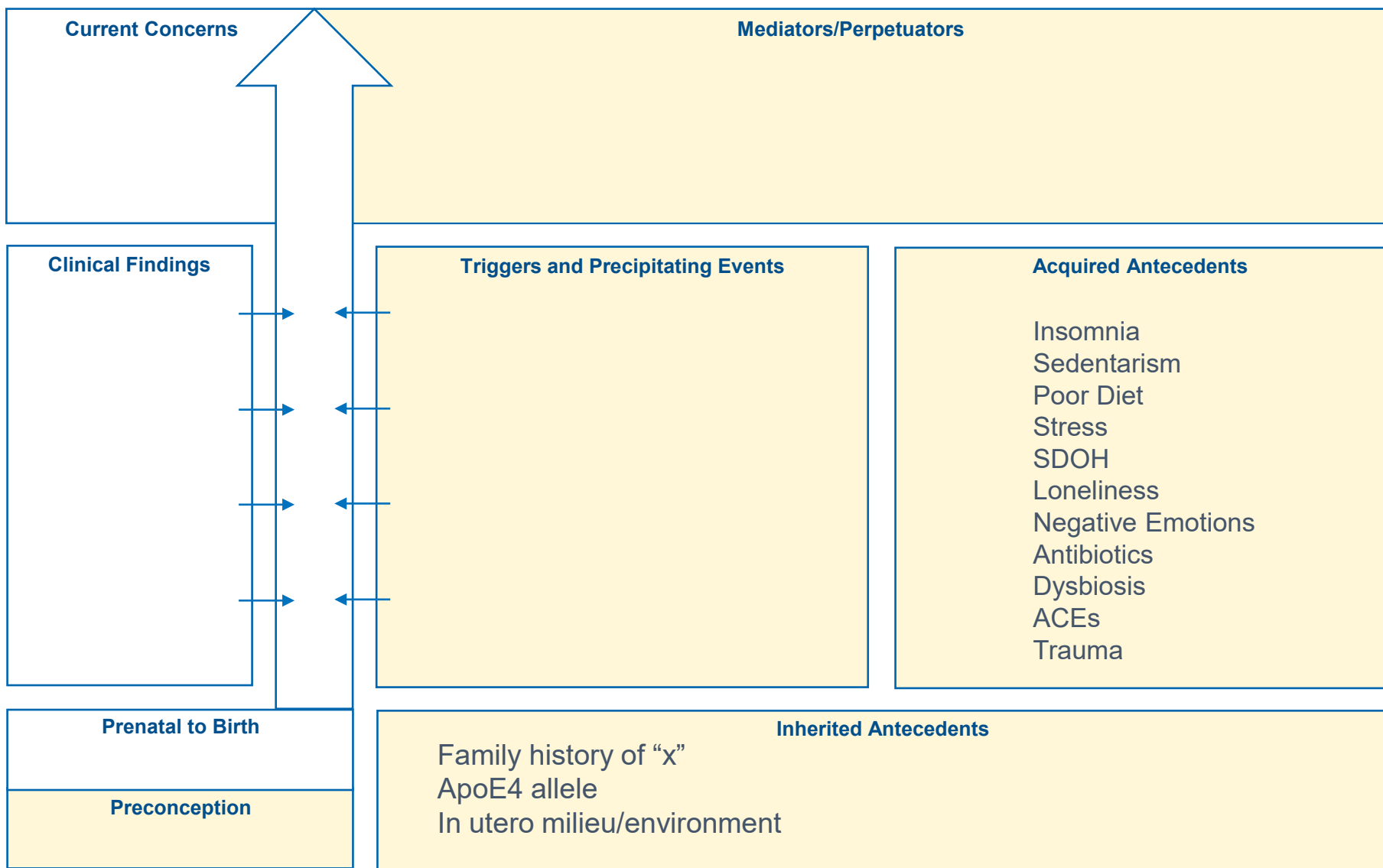
Acquired Antecedents

Examples of acquired antecedents:

Insomnia
Sedentarism
Poor Diet
Stress
Social Determinants of Health
Loneliness

Negative Emotions
Antibiotics/Dysbiosis
Diet
Lifestyle
Gut
Psycho-social-spiritual

Functional Medicine Timeline



Representative
Examples of
Most Common
Antecedents

Long Description

- The Functional Medicine timeline with the acquired antecedents and inherited antecedents sections filled out.
- Acquired antecedents
 - Insomnia
 - Sedentarism
 - Poor diet
 - Stress
 - S D O H
 - Loneliness
 - Negative emotions
 - Antibiotics
 - Dysbiosis
 - ACE's
 - Trauma
- Inherited antecedents
 - Family history ox "x"
 - Apo E 4 allele
 - In utero milieu/environment

Triggers

Triggers are single, episodic, or recurrent exposures, factors, or life events that activate(d) physiological dysfunction and/or symptoms within seconds to weeks.

Triggers

Examples of triggers:
**Allergens, Accidents, Trauma,
Surgeries, Environmental Exposures,
Infections**

Many triggers can be avoided or eliminated.
Examples: Triggering foods and allergens

If physiological dysfunction is addressed,
the effects of triggers can be mitigated.
Ex: Food sensitivities/repair leaky gut

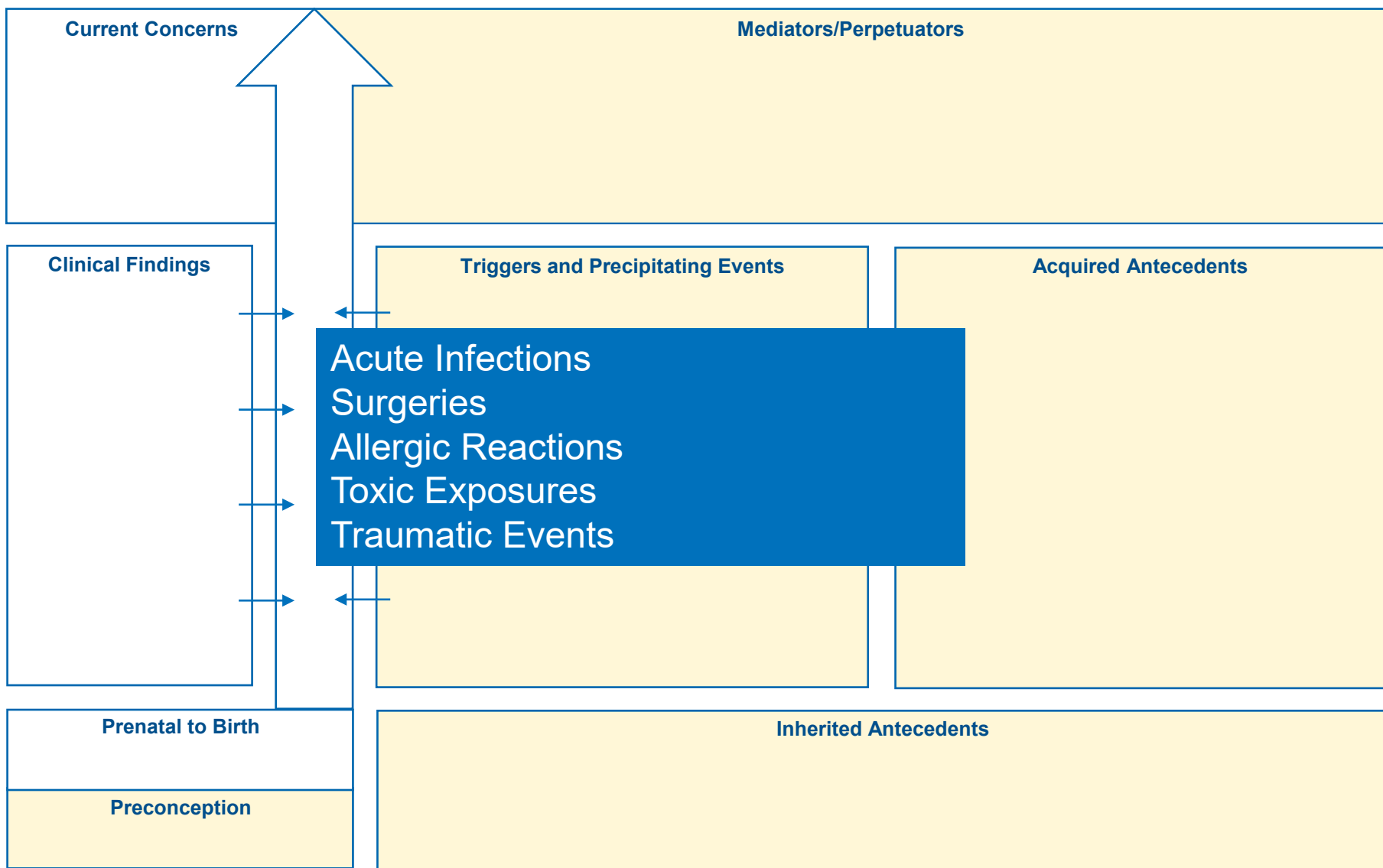
Triggering (Precipitating) Life Events

“My health was never the same since...”

“I have never felt well since...”

Precipitating/triggering life events can be reframed, and their effects on physiological function can be mitigated.

Functional Medicine Timeline



Representative
Examples of
Most Common
Triggers

Long Description

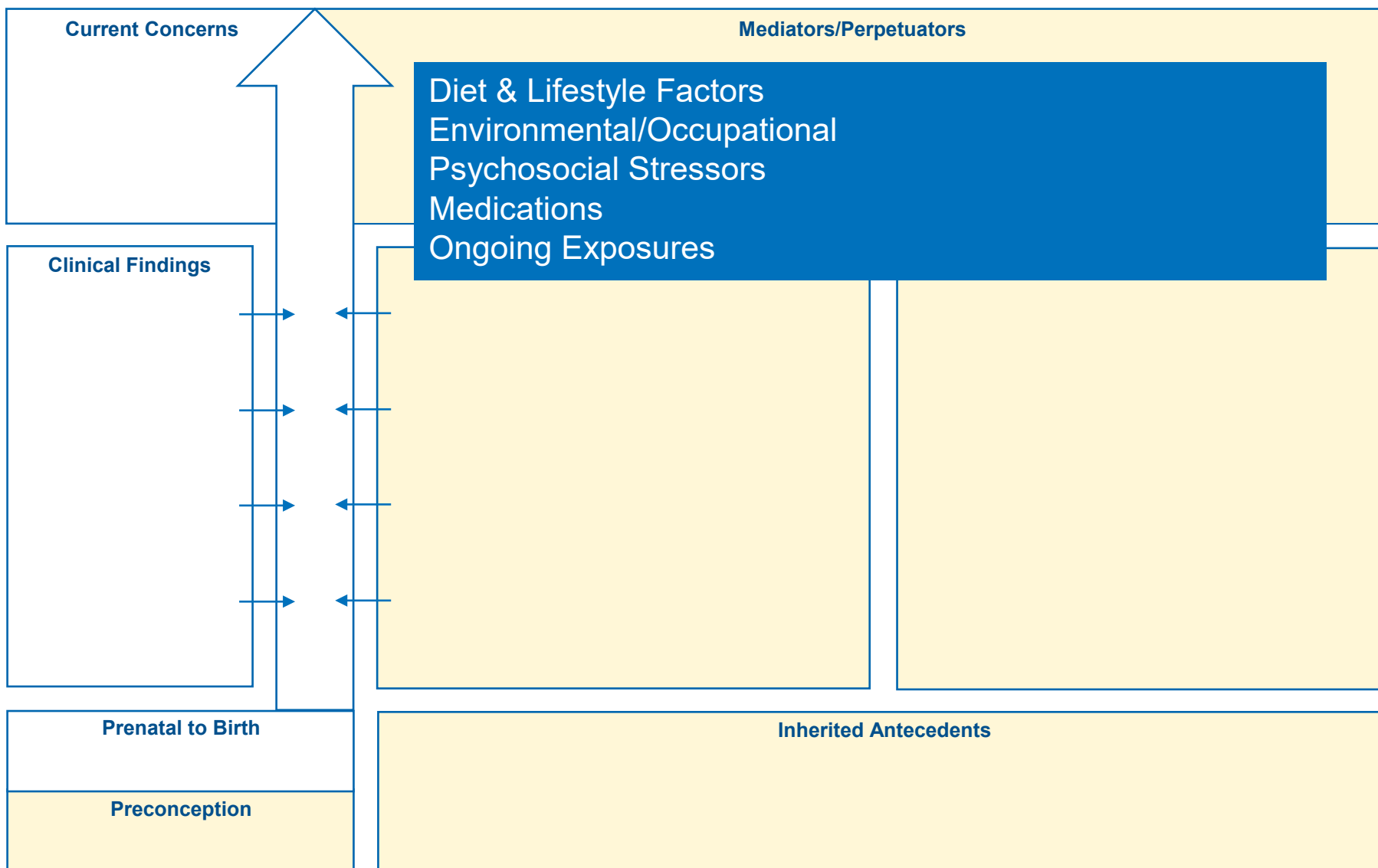
- The Functional Medicine timeline, with the triggers and precipitating events section filled out.
- Triggers and precipitating events
 - Acute infections
 - Surgeries
 - Allergic reactions
 - Toxic exposures
 - Traumatic events

Mediators

FACTORS of EXCESS or DEFICIENCY
Most often diet & lifestyle

Mediators are current and ongoing biochemical, environmental, dietary, lifestyle, or psychosocial factors that contribute to and perpetuate symptoms, physiological dysfunction, and/or disease and deter the restoration of normal function and well-being.

Functional Medicine Timeline



Representative
Examples of
Most Common
Mediators

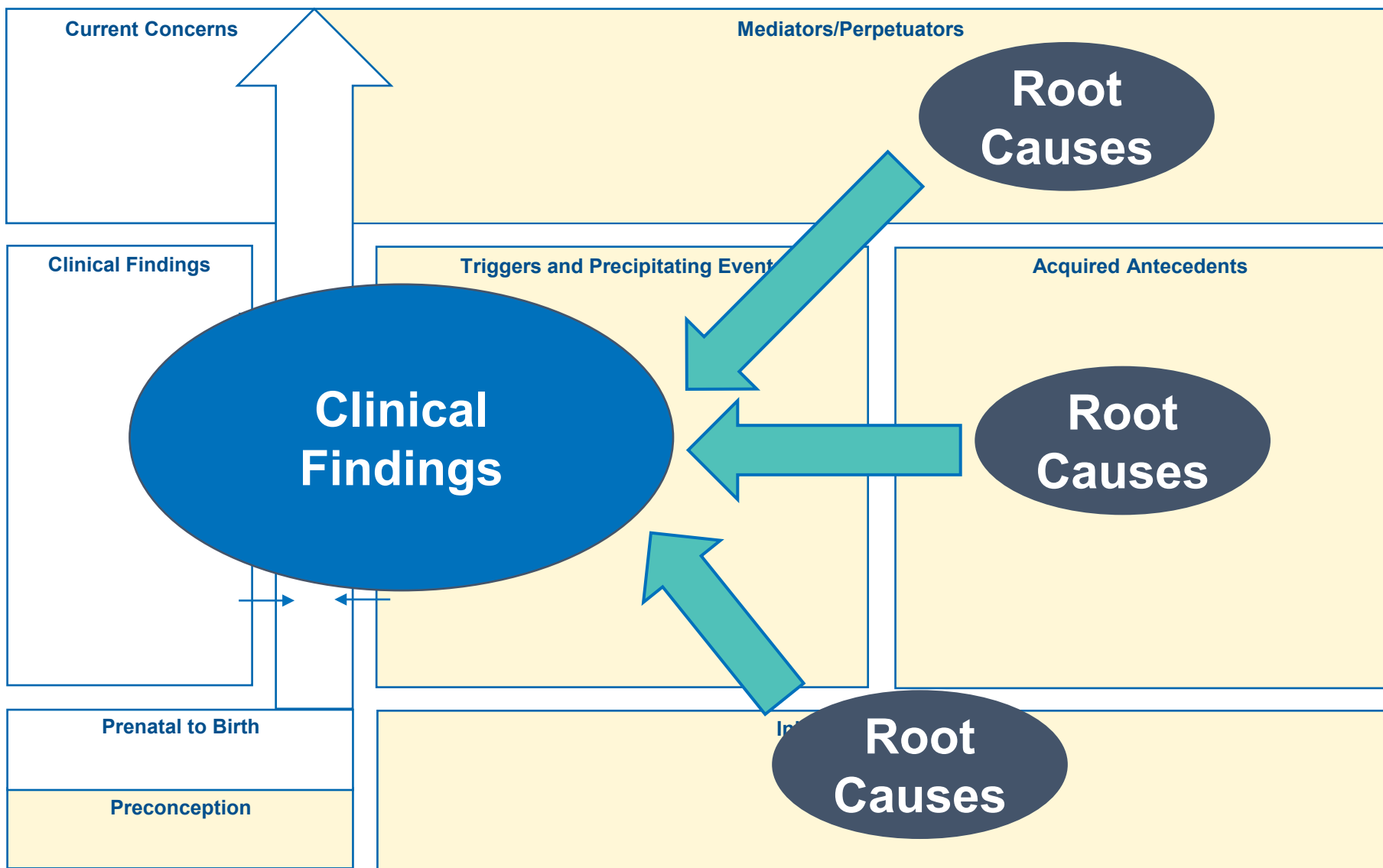
Long Description

- The Functional Medicine timeline with the mediators and perpetrators section filled out.
 - Diet and lifestyle factors
 - Environmental/occupational
 - Psychosocial stressors
 - Medications
 - Ongoing exposures

Common Antecedents, Triggers, and Mediators

<u>Antecedents</u>	<u>Triggers</u>	<u>Mediators</u>
• Congenital • Demographic • Dietary • Habitual • Environmental • Occupational • Learned • Traumatic • Disease-Induced • Drug-Induced	• Trauma • Microbes • Antigens • Environmental Toxins • Radiation • Social Interactions • Memories • Diverse Sensory Stimuli	• Hormones, Cytokines, etc. • Inflammation • Reactive Oxygen Species • Metabolites • Thoughts • Beliefs • Social Reinforcement • Classical Conditioning • Lifestyle Behaviors • Dietary Practices

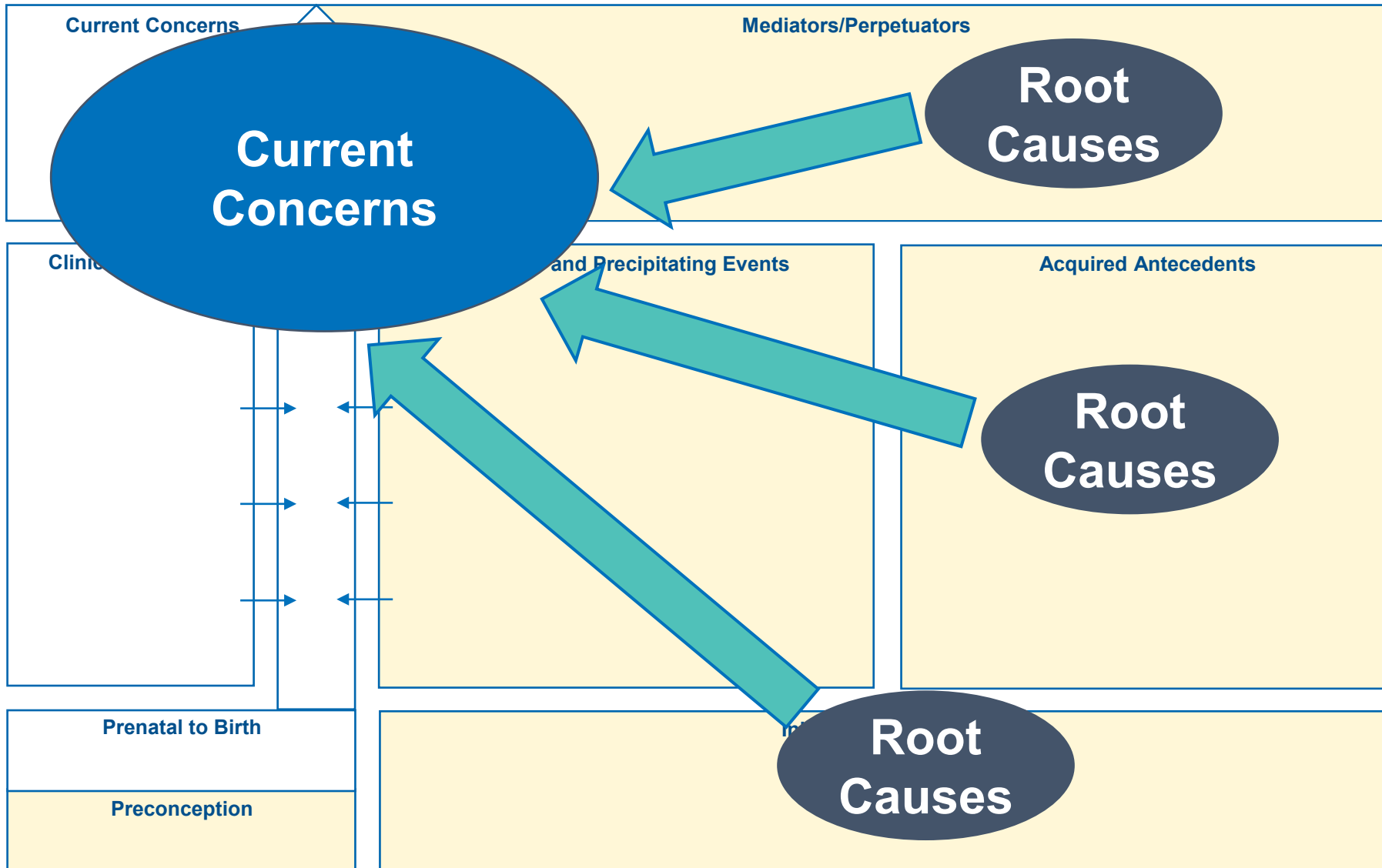
Functional Medicine Timeline



Long Description

- The Functional Medicine timeline identifying mediators/perpetuators, acquired antecedents, and inherited antecedents as root causes of the clinical findings.

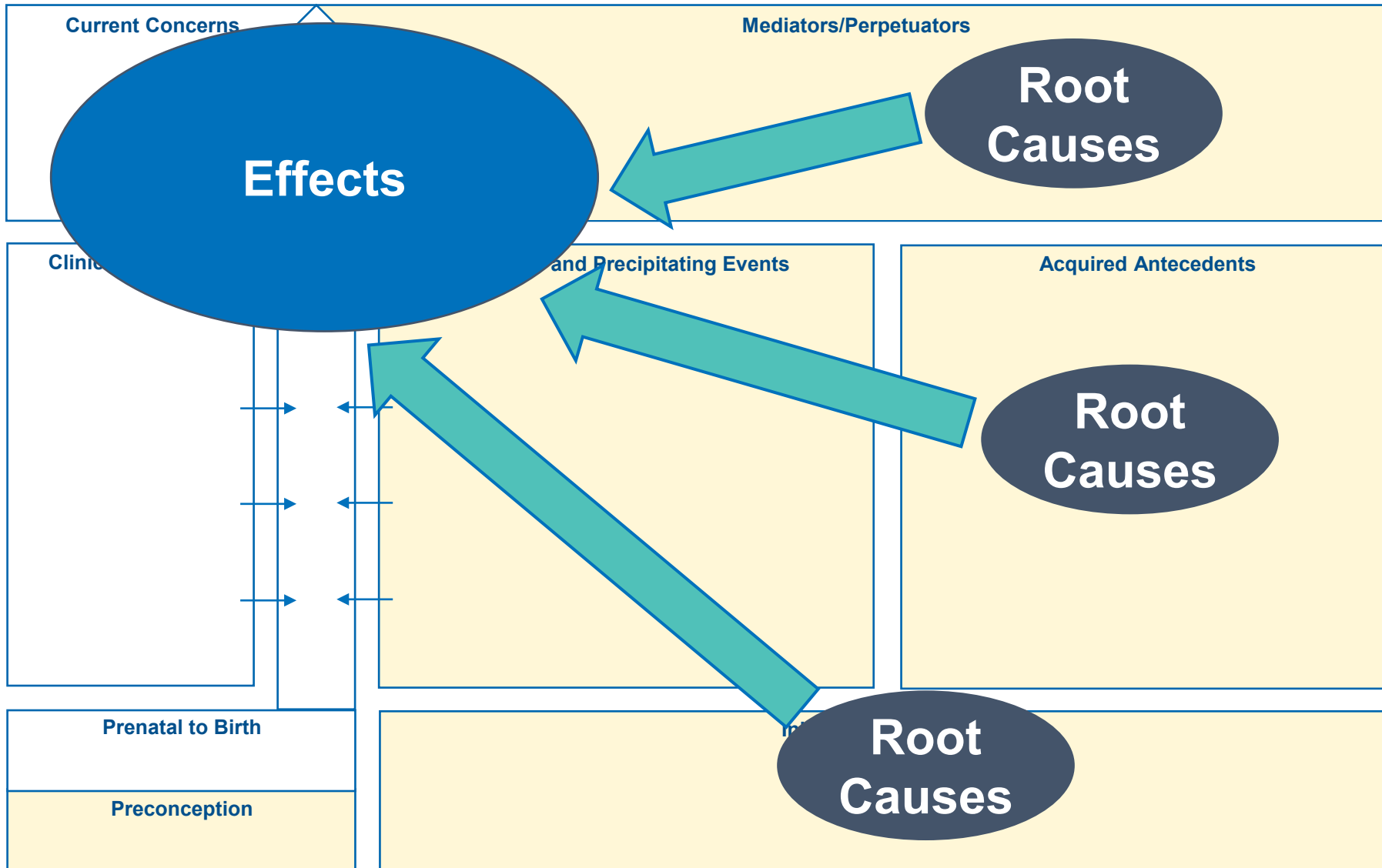
Functional Medicine Timeline



Long Description

- The Functional Medicine timeline identifying mediators/perpetuators, acquired antecedents, and inherited antecedents as root causes of the current concerns

Functional Medicine Timeline



Long Description

- The Functional Medicine timeline identifying mediators/perpetuators, acquired antecedents, and inherited antecedents as root causes of the effects on the individual.

Populating Sections of the Timeline

- Antecedents, Triggers, and Mediators represent the ROOT CAUSES of physiological dysfunction.
- Clinical Findings and Current Concerns represent the phenotypic EFFECTS of the physiological dysfunction caused by antecedents, triggers, and mediators.
- Root cause medicine must therapeutically address individualized antecedents, triggers, and mediators.

Clinical Focus

- IFM Toolkit items designed to gather antecedents, triggers, mediators, clinical findings, and effects.



Adult Intake Questionnaire

This IFM questionnaire includes conventional intake questions/FM factors that are stated on the clinical focus slide (ATMs).



Male Intake Questionnaire

General Information

Name _____ Age _____
Date of Birth _____ Email _____
Address _____ City _____
State _____ Zip _____
Phone (Home) _____ (Cell) _____
Genetic Background: African American Hispanic Mediterranean
Native American Caucasian Northern European
Other _____
When, where and from whom did you last receive medical or health care?

Emergency Contact: _____ Relationship _____
(Cell) _____ (Work) _____
How did you hear about our practice?
Clinic website IFM website ☐ Referral from doctor ☐ Referral from friend/family member
Other _____

Current Health Concerns

Please rank current and ongoing health concerns in order of priority

Describe Problem	Severity	Prior Treatment/Approach
Example: Post Nasal Drip	Mild ☐ Moderate ☐ Severe ☐	Elimination Diet
	☐ Mild ☐ Moderate ☐ Severe	
	☐ Mild ☐ Moderate ☐ Severe	
	☐ Mild ☐ Moderate ☐ Severe	
	☐ Mild ☐ Moderate ☐ Severe	
	☐ Mild ☐ Moderate ☐ Severe	
	☐ Mild ☐ Moderate ☐ Severe	
	☐ Mild ☐ Moderate ☐ Severe	
	☐ Mild ☐ Moderate ☐ Severe	
	☐ Mild ☐ Moderate ☐ Severe	
	☐ Mild ☐ Moderate ☐ Severe	
	☐ Mild ☐ Moderate ☐ Severe	

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Female Intake Questionnaire

General Information

Name _____ Age _____ Today's Date _____
Date of Birth _____ Email _____
Address _____ City _____
State _____ Zip _____
Phone (Home) _____ (Cell) _____ (Work) _____
Genetic Background: African American Hispanic Mediterranean Asian
Native American Caucasian Northern European
Other _____
When, where and from whom did you last receive medical or health care?

Emergency Contact: _____ Relationship _____ Phone (Home) _____
(Cell) _____ (Work) _____
How did you hear about our practice?
Clinic website IFM website ☐ Referral from doctor ☐ Referral from friend/family member ☐ Social media
Other _____

Current Health Concerns

Please rank current and ongoing health concerns in order of priority

Describe Problem	Severity	Prior Treatment/Approach	Success
Example: Post Nasal Drip	Mild ☐ Moderate ☐ Severe ☐	Elimination Diet	Good ☐ Fair ☐ Excellent ☐
	☐ Mild ☐ Moderate ☐ Severe		☐ Good ☐ Fair ☐ Excellent
	☐ Mild ☐ Moderate ☐ Severe		☐ Good ☐ Fair ☐ Excellent
	☐ Mild ☐ Moderate ☐ Severe		☐ Good ☐ Fair ☐ Excellent
	☐ Mild ☐ Moderate ☐ Severe		☐ Good ☐ Fair ☐ Excellent
	☐ Mild ☐ Moderate ☐ Severe		☐ Good ☐ Fair ☐ Excellent
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	☐ Mild ☐ Moderate ☐ Severe		☐ Good ☐ Fair ☐ Excellent
	☐ Mild ☐ Moderate ☐ Severe		☐ Good ☐ Fair ☐ Excellent
	☐ Mild ☐ Moderate ☐ Severe		☐ Good ☐ Fair ☐ Excellent
	☐ Mild ☐ Moderate ☐ Severe		☐ Good ☐ Fair ☐ Excellent
	☐ Mild ☐ Moderate ☐ Severe		☐ Good ☐ Fair ☐ Excellent

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Version 4

Long Description

- I F M Toolkit Items: Male and Female intake questionnaires. Accessible version available in the course materials.

Diet, Nutrition, & Lifestyle Journal

- Can be very valuable in the gather step.
- Will be fully addressed in a future presentation in this course.



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Diet, Nutrition & Lifestyle Journal—7 Day

Patient Name

Date

Food Plan Type

Day 1

Day Event	Food & Drink Intake (include type, amount, brand)	Macronutrients (PFC) and Phytonutrients
Rising Time		F C ☐ R ☐ O ☐ Y ☐ G ☐ B/P/BL ☐ W/T/BR
Breakfast Time		F C ☐ R ☐ O ☐ Y ☐ G ☐ B/P/BL ☐ W/T/BR
Mid-AM Snack Time		F C ☐ R ☐ O ☐ Y ☐ G ☐ B/P/BL ☐ W/T/BR
Lunch Time		F C ☐ R ☐ O ☐ Y ☐ G ☐ B/P/BL ☐ W/T/BR
Mid-PM Snack Time		F C ☐ R ☐ O ☐ Y ☐ G ☐ B/P/BL ☐ W/T/BR
Dinner Time		F C ☐ R ☐ O ☐ Y ☐ G ☐ B/P/BL ☐ W/T/BR
PM Snack Time		F C ☐ R ☐ O ☐ Y ☐ G ☐ B/P/BL ☐ W/T/BR
Bed Time		F C ☐ R ☐ O ☐ Y ☐ G ☐ B/P/BL ☐ W/T/BR

P: Proteins; F: Fats; C: Carbohydrates; R: Red; O: Orange; Y: Yellow; G: Green; B/P/BL: Blue/Purple/Black; W/T/BR: White/Tan/Brown

Sleep & Relaxation	Exercise & Movement	Stress	Relationships
Sleep Quantity: (hrs)	Type, Duration, & Intensity ☐ Aerobic:	Stress reduction practices:	Supporting:
Quality: ☐ Poor ☐ Fair ☐ Good			
Relaxation ☐ Yes ☐ No	☐ Strength:	Stressors:	Non-supporting:
Type/Amount: 	☐ Flexibility:		

Mental	Emotional	Spiritual

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Version 4; updated 2024

Long Description

- I F M Toolkit Item: Diet, Nutrition, and Lifestyle Journal – 7 day. Accessible version available in the course materials.

Medical Symptoms Questionnaire

- Not validated
- No standard reference range
- Clinical utility is to compare intra-individual scores at subsequent visits
- Inter-individual comparisons lack clinical utility

Accessible version in your toolkit.

Medical Symptoms Questionnaire (MSQ)

Patient Name Date

Instructions

Use this point scale to rate each of the following symptoms based on your typical health profile for the past 14 days:

0 = Never or almost never have the symptom	3 = Frequently have it; effect is not severe
1 = Occasionally have it; effect is not severe	4 = Frequently have it; effect is severe
2 = Occasionally have it; effect is severe	

Head

- ☐ Headaches
- ☐ Faintness
- ☐ Dizziness
- ☐ Insomnia
- ☐ Total

Eyes

- ☐ Watery or itchy eyes
- ☐ Swollen, reddened, or sticky eyelids
- ☐ Bags or dark circles under eyes
- ☐ Blurred or tunnel vision (doesn't include nearsightedness or farsightedness)
- ☐ Total

Ears

- ☐ Itchy ears
- ☐ Earaches, ear infections
- ☐ Drainage from ear
- ☐ Ringing in ears, hearing loss
- ☐ Total

Nose

- ☐ Stuffy nose
- ☐ Sinus problems
- ☐ Hay fever
- ☐ Sneezing attacks
- ☐ Excessive mucus formation
- ☐ Total

Please continue on the next page

Version 3

Long Description

- I F M Toolkit Item: Medical Symptoms Questionnaire (M S Q).
Accessible version available in the course materials.

Other Questionnaires

Patient-Reported Outcomes Measurement Information System (PROMIS):

- A set of questionnaires developed by the National Institutes of Health (NIH) to measure physical, mental, and social health outcomes (validated).
- Designed to be used in clinical research and practice to assess how patients experience their health and well-being.

<https://commonfund.nih.gov/patient-reported-outcomes-measurement-information-system-promis>



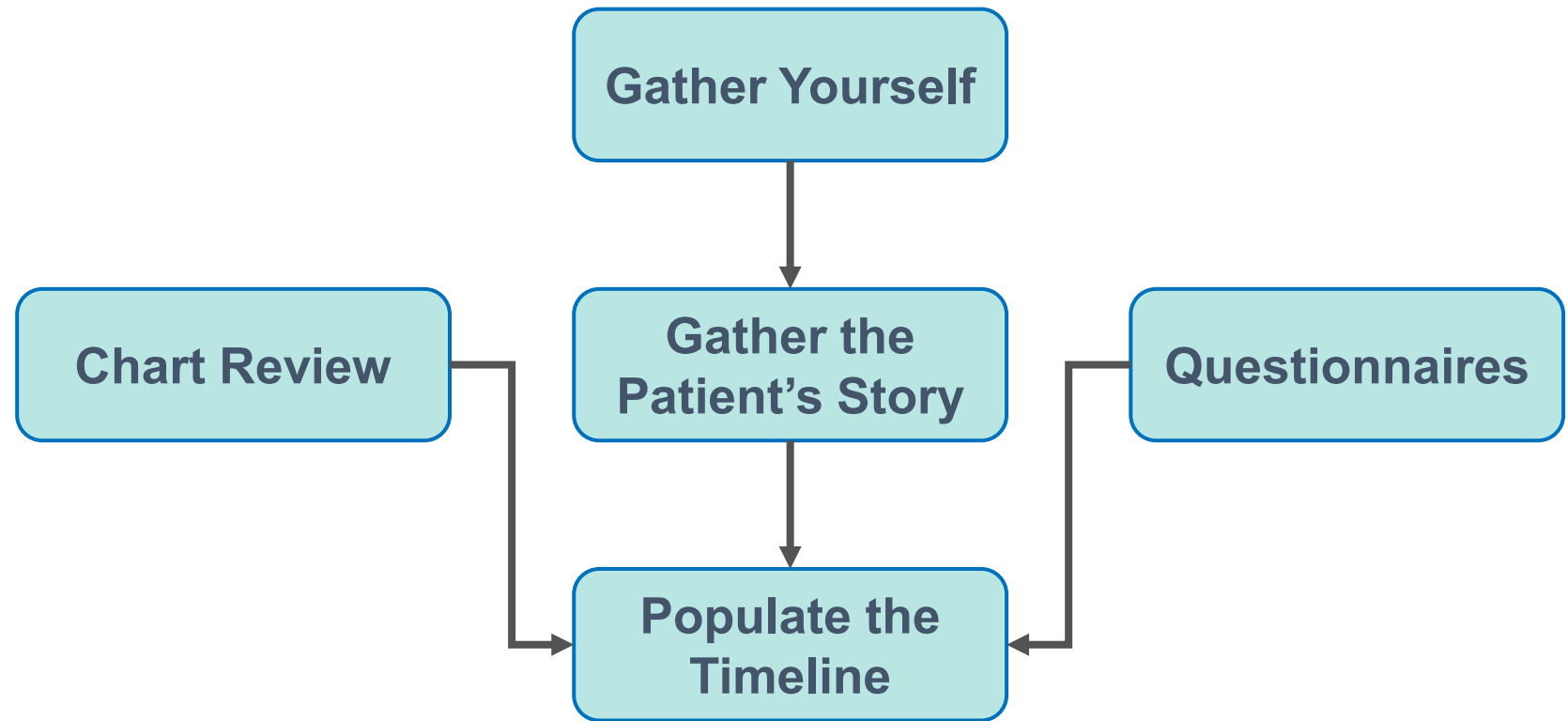
Clinical Focus

- Summary, conclusion, and key clinical takeaways.
- Clinical flow paths for the GATHER step of GOTOIT.



Key Clinical Takeaway:

Flow Path for the Gather Step of GOTOIT

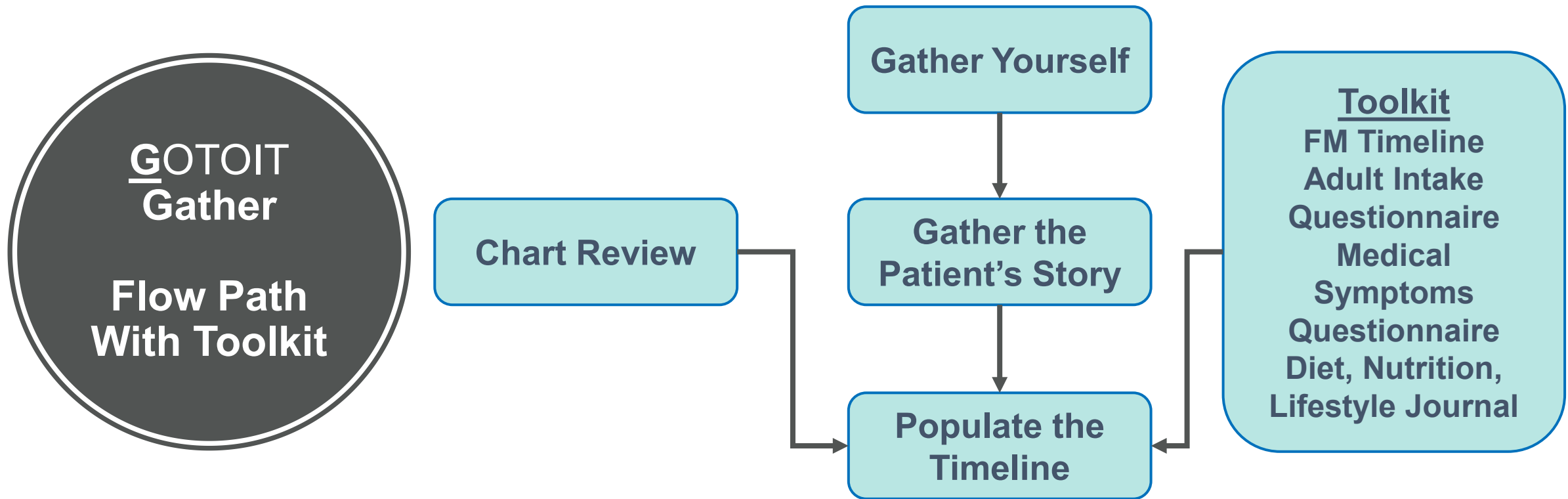


Long Description

- Gather yourself, gather the patient's story, questionnaires, and chart review to populate the patient timeline.

Key Clinical Takeaway:

Flow Path for the Gather Step of GOTOIT (With Toolkit Items)



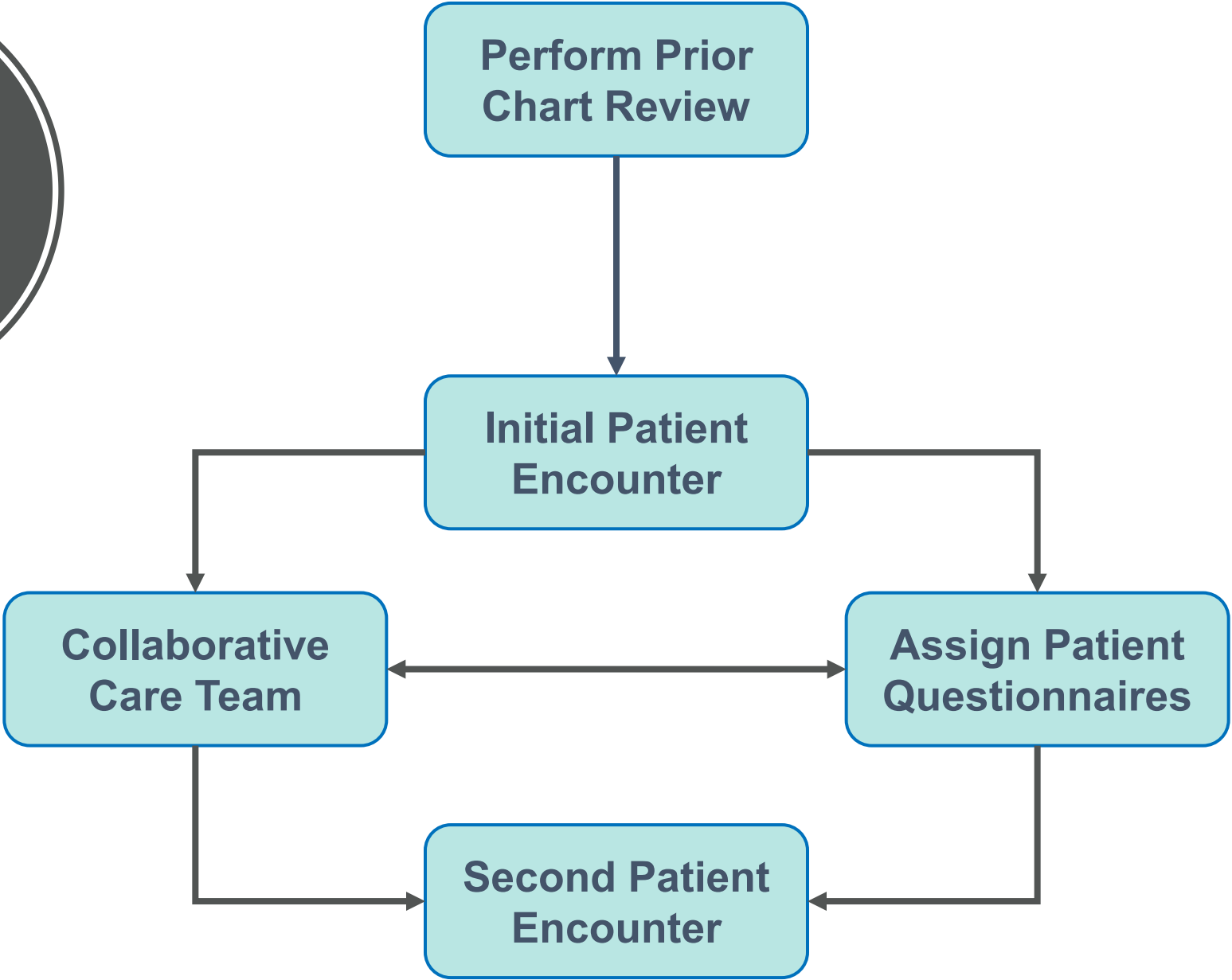
Long Description

- Toolkit items used to populate the patient's timeline:
 - F M timeline
 - Adult intake questionnaire
 - Medical symptoms questionnaire
 - Diet, Nutrition, and Lifestyle Journal

Key Practice Takeaway: Clinical Flow Paths for the Gather Step of GOTOIT

OPTION #1
Gather

**Clinical
Flow Path**

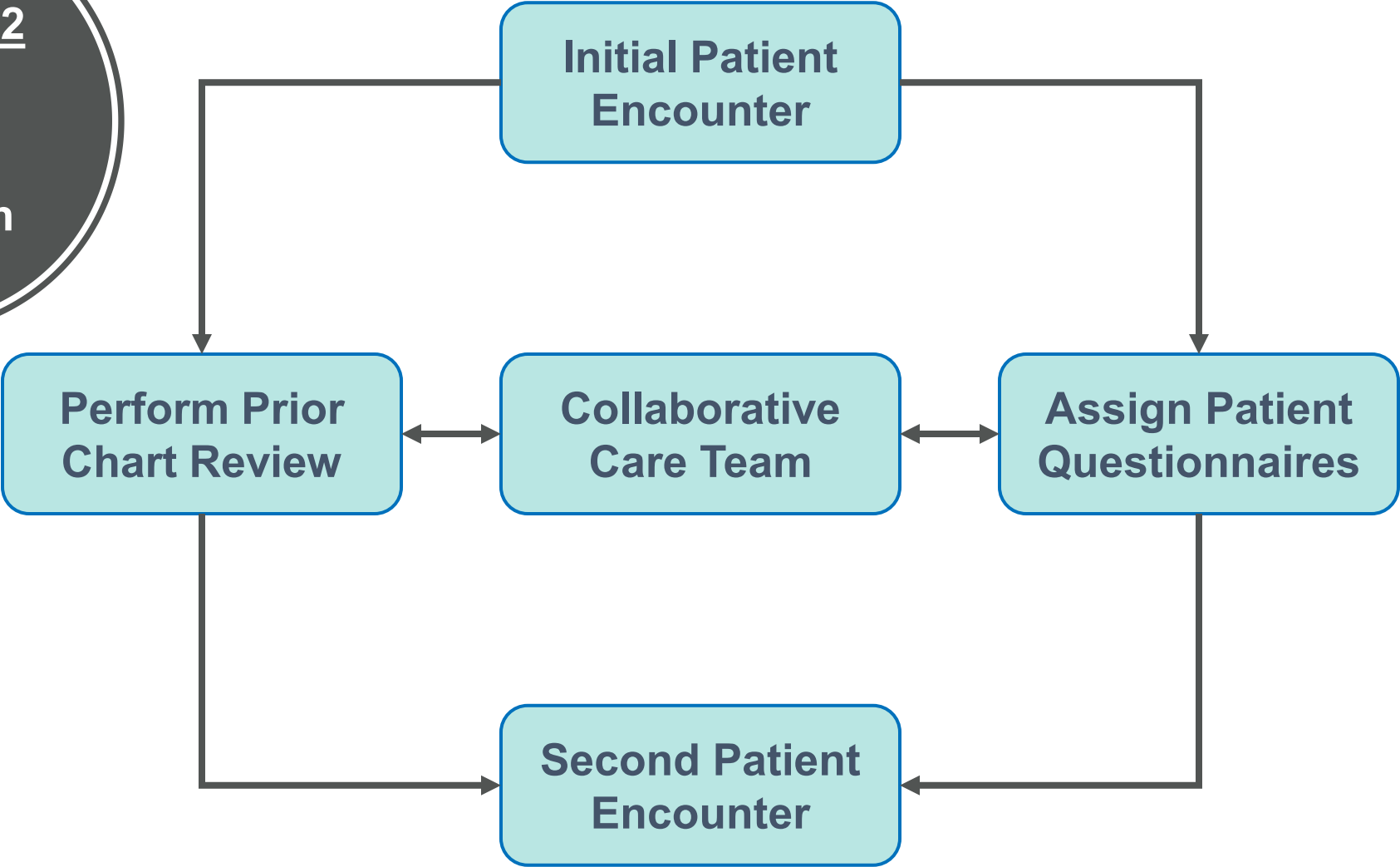


Long Description

- Clinical flowpath option number 1
 1. Perform prior chart review
 2. At the initial encounter, assign patient questionnaires and utilize collaborative care team
 3. Use the above to inform the second patient encounter

OPTION #2
Gather

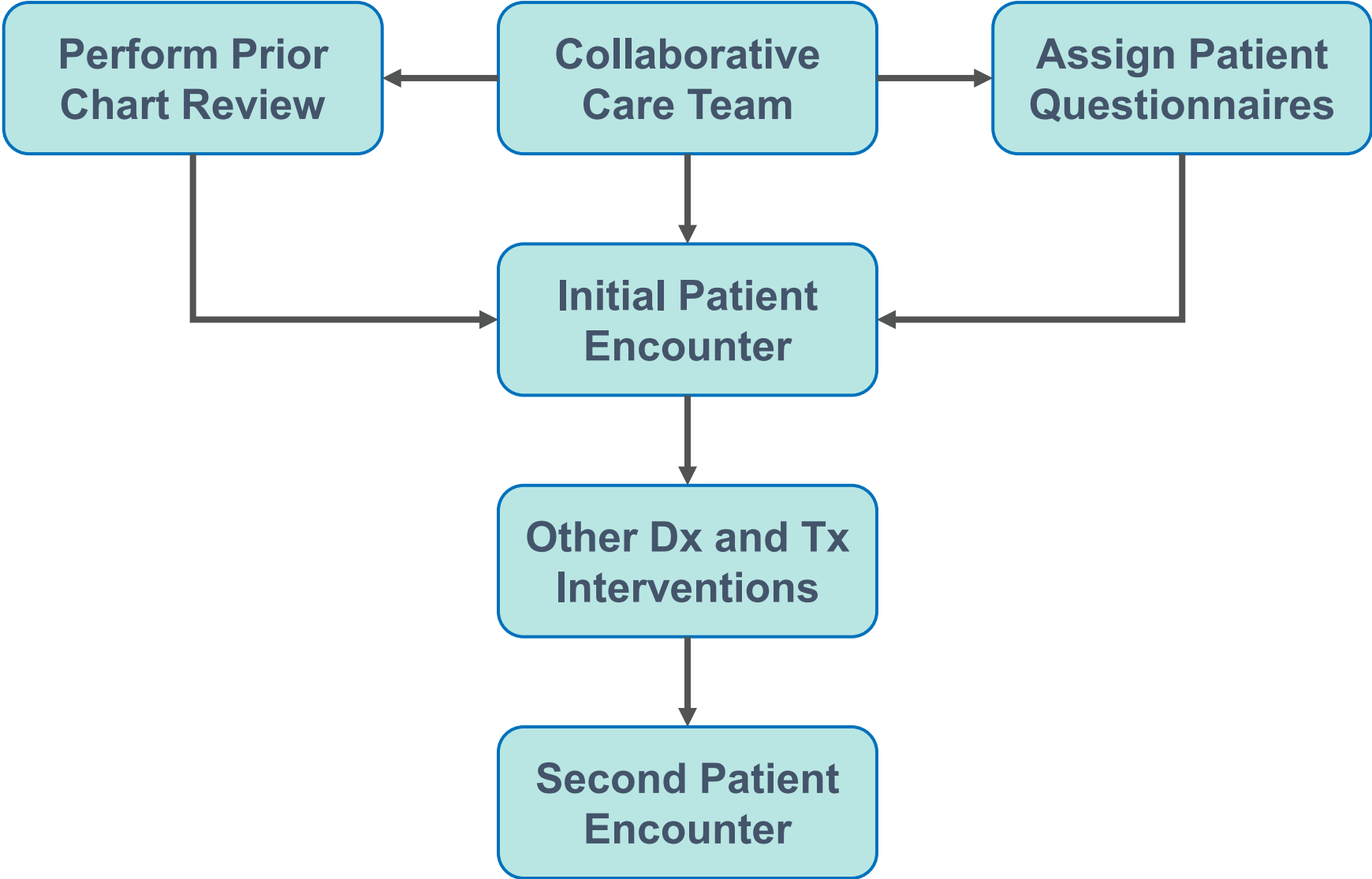
**Clinical
Flow Path**



Long Description

- Clinical flowpath option number 2
 1. Initial patient encounter
 2. Perform prior chart review, assign patient questionnaires and utilize collaborative care team
 3. Use the above to inform the second patient encounter

OPTION #3
Gather
Clinical
Flow Path



Long Description

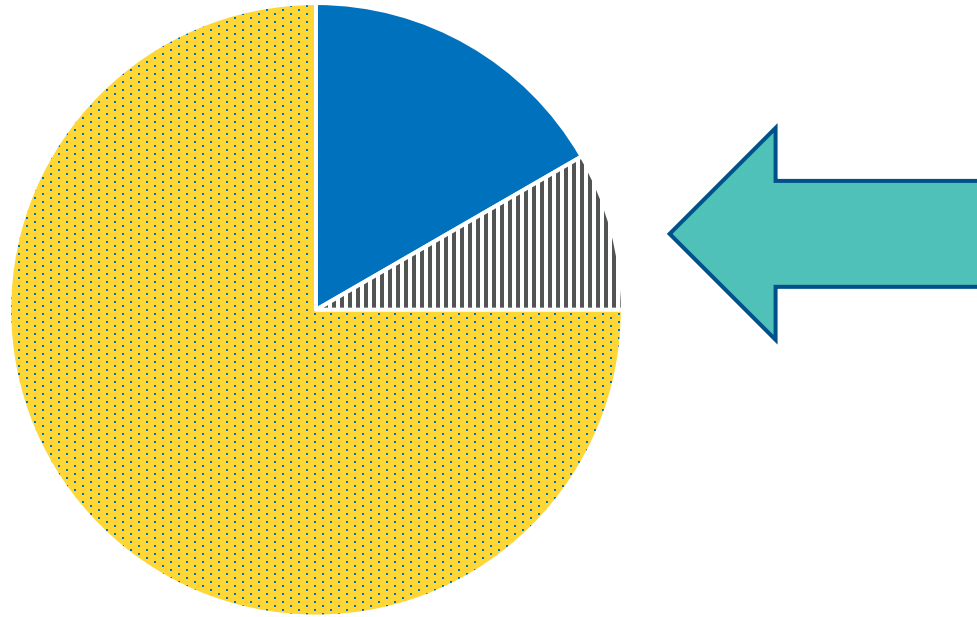
- Clinical flowpath option number 3
 1. Perform prior chart review, assign patient questionnaires and utilize collaborative care team
 2. Meet for the initial encounter
 3. Other diagnoses and treatment interventions
 4. Use the above to inform the second patient encounter

Clinical Focus

- Key clinical questions at the point of care to efficiently and effectively gather the patient's clinical findings, antecedents, triggers, and mediators.



**How can you incorporate the
functional medicine clinical
method for 5 minutes of a
15-minute visit?**



15-Minute Approach: The Functional Medicine Timeline

What are the key questions to ask that will elicit the most important clinical findings, antecedents, triggers, and mediators of a patient's life in order to begin populating the timeline in just a few minutes?

Gathering the Story on the Timeline: Rapid Identification of Clinical Findings

- What are your current concerns and symptoms?
- What were your health challenges and diagnoses:
 - As a child?
 - As a teen?
 - As an adult?
- What surgeries have you had?
- What medications and supplements do you take?
- What lab and diagnostic test results have been worrisome?

Gathering the Story on the Timeline: Rapid Identification of Antecedents

- What conditions affect your family members or run in your family?
- What do you know about your mother's pregnancy with you?
- Were you born by C-section? Were you breastfed? For how long?
- How often have you received antibiotics?
- What factors in your past home or work environment have affected your health?
- What else happened in the past that you think affects your health now?

Gathering the Story on the Timeline: Rapid Identification of Triggers

- What events/environments/circumstances in your past most affected your health or made you feel worse immediately?
- What do you avoid now because it makes you feel worse?
- What do you do because it makes you feel better?
- How would you complete these sentences:
 - “My health has never been the same since _____.”
 - “My health became worse after _____.”

Gathering the Story on the Timeline: Rapid Identification of Mediators

- What factors in your home or work environment affect your health?
- Which of your regular habits, behaviors, and pastimes:
 - Have a negative effect on your health?
 - Have a positive effect on your health?
- How would you complete these sentences:
 - “My health would be better if I spent less time _____.”
 - “My health would be better if I spent more time _____.”

GATHER:

*The Patient's Story on the Functional Medicine
Timeline*

APPLYING FUNCTIONAL MEDICINE IN CLINICAL PRACTICE